

Name: Daniel S May | DOB: 10/11/1989 | MRN: 10105654130 | PCP: Joshua W Clark, NP | Legal Name:
Daniel S May

Office Visit - Oct 31, 2025

with Gregory Krause, MD at Mass Eye and Ear Otolaryngology Clinic



Notes from Care Team



Progress Notes by Gregory Krause, MD at 10/31/2025 12:45 PM



Mass General Brigham
Mass Eye and Ear



HARVARD
MEDICAL SCHOOL

MASSACHUSETTS EYE AND EAR INFIRMARY DEPARTMENT--OTOLARYNGOLOGY

Date: 10/31/2025

Patient: Daniel S May DOB: 10/11/1989 MEEI MRN: 8026121

PCP: Pcp, Not Required

History of Present Illness

Daniel S May is a 36 year old male who presents with left ear fullness and sinus issues.

He experiences a persistent sensation of fullness in his left ear, which has been bothersome and affects his sleep and daily life. A hearing test was performed, and the patient recalls being told there was pressure in the left ear. He describes the ear as feeling 'stuck' and 'sticky'.

He has a history of chronic ear infections over the past ten years, occurring every few months, with worsening symptoms over the past year. He has been using treatments such as Mucinex, nasal sprays, and steroid sprays like budesonide, which provided short-term relief. Nasal rinses have helped clear thick, sticky mucus from his sinuses, sometimes containing blood. Despite these efforts, symptoms persist, particularly in his ears when blowing his nose.

His social history includes working in real estate, which does not involve exposure to noisy environments. He denies frequent exposure to loud concerts or firearms, which could contribute to hearing loss. He is currently seeking new employment due to the impact of his symptoms on his previous job.

Patient: DANIEL MAY (PMRN:10105654130)

Get Report

☐ Left ☐ Right ☒ Air

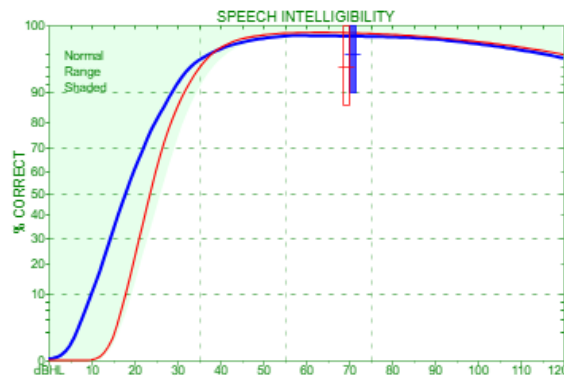
DOB: 10/11/1989 Age: 35 y.o. DOS: 10/4/2025

☒ Bone ☒ SF

Audiologist: Anna M. Keenan Au.D., CCC-A Ref: KUSH PANARA, M.D.

Assist:

Clinic: Audiology

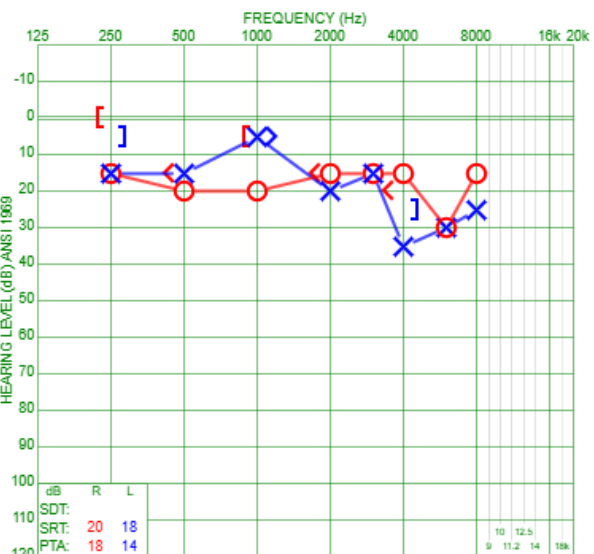


SPEECH TESTING					
RIGHT			LEFT		
LIST	dBHL	Score	LIST	dBHL	Score
W22 3A	70	98%	W22 4A	70	98%

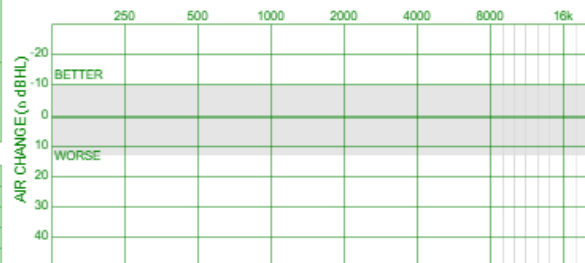
SYMBOLS	RIGHT	LEFT
Speech Intelligibility Index Prediction		
Speech Score 95%CD		
Air Conduction		
AC Masked		
Bone Conduction		
BC Masked		
Sound Field		
Response at Limit		
No Response		
Acoustic Reflex Ipsilateral Probe		
Acoustic Reflex Contralateral Probe		

TYMPANOMETRY			
FEATURE	RIGHT	LEFT	
VOLUME	1.0 ml	0.7 ml	
PRESSURE	-5 daPa	-181 daPa	
MOBILITY	2.2 ml	0.8 ml	
SHAPE	Normal	Normal	

REFLEX DECAY		
FREQUENCY	RIGHT	LEFT
500 Hz		
1000 Hz		
2000 Hz		
4000 Hz		



LowF-PTA (dB): 18 12 HighF-PTA (dB): 19 26



REASON FOR TEST

Eustachian tube dysfunction

HISTORY

Patient is seen for suspected eustachian tube dysfunction. He reports aural fullness and popping bilaterally, left ear worse. Patient has a history of sinus surgeries. He denies additional otologic complaints.

HEARING MEASUREMENT

TDH and insert earphones were used for testing. Ipsilateral acoustic reflexes to broadband stimulation are absent in the right ear.

IMPRESSIONS

Tympanometry results are consistent with normal middle ear pressure and hypermobility in the right ear, and negative middle ear pressure with normal mobility in the left ear. Conductive overlay is noted at 250 and 1000 Hz in the right ear. Absence of acoustic reflex suggests the conductive components are of middle ear origin. The patient may experience communication difficulties in adverse listening situations, and is expected to benefit from the use of good communication strategies.

CONSULTATION

Results were reviewed with the patient.

10/3/25 office visit-

Daniel S May is a 35 year old male with chronic sinus issues who presents with bilateral ear fullness and eustachian tube dysfunction. He was referred by Dr. Gray for treatment of otologic symptoms.

He has a history of chronic sinus issues and has undergone two septoplasty procedures over the past ten years. He experiences chronic ear infections approximately every month or two, which have worsened over the past year. His current symptoms include ear fullness, popping, and crackling, particularly in the left ear. He describes a sensation of a sticky, sappy substance that he feels may be present in his eustachian tubes. He has been using budesonide nasal rinses, which have cleared his sinuses but not resolved the ear symptoms. He has been on oral prednisone in the past, which also did not alleviate his ear symptoms. He experiences occasional migraines, with two severe episodes in the past few months, but denies frequent ear pain. He has not flown recently but recalls experiencing vertigo after a flight ten years ago. No autophony.

ASSESSMENT/PLAN

Bilateral aural fullness

Symptoms greater in left ear. Most likely secondary to eustachian tube dysfunction given concurrent sinus issues and lack of autophony, TMJ, Migraine, or vertiginous symptoms. Will obtain audiogram/tympanogram. Discussed with Mr. May that if negative pressure noted on tympanogram would recommend myringotomy tube insertion in left ear. If no negative pressure, would recommend a diagnostic tympanostomy and if aural fullness symptoms resolved than perform a subsequent PE tube insertion.

9/17/25 Rhinology visit-

Impression:

1. Rhinitis medicamentosa - improved.
2. Chronic rhinosinusitis.
3. Inferior turbinate hypertrophy.

Plan:

Mr. May and I reviewed his history and symptoms and the findings on nasal endoscopy. Overall his nasal symptoms have improved since stopping the hydrogen peroxide in his rinses and minimizing the Afrin and starting the Budesonide irrigations. I stressed the importance of complete cessation of all topical nasal decongestant sprays. He can continue the Budesonide irrigations and can try to decrease to daily. I do think that allergy evaluation will be useful for him and we will help arrange this. Most of his symptoms are currently ear related and we will try to help expedite his Comprehensive ORL evaluation for this.

8/11/25 MEE ED visit-

Daniel S May (MRN 8026121) is a 35 y.o. male with PMH including CRS

s/p sinus surgery x2 >10 years ago. Was seen in MEE ED 07/16/25 with months of sinus congestion and nasal obstruction. Started on inhaled steroid nasal sprays and referred to Rhinology.

Had scheduling errors and returns to the ED for follow up.

Symptoms include thick nasal discharge, facial pressure,

Has had recurrent sinus infections in the past.

Has used saline irrigations, flonase without improvement.

ASSESSMENT

CRSwoNP- CT obtained today showing bilateral sinus disease.

Recommended starting nasal saline irrigations and flonase BID. Referral to Rhinology

Left ear clicking- differential includes ETD vs middle ear myoclonus. Obtain audiogram, follow up with neurotology

EXAMINATION

There were no vitals taken for this visit.

GENERAL APPEARANCE: well developed, well nourished patient in no acute distress.

VOICE: normal

BREATHING: normal, not labored, no stridor

FACE: no lesions

FACIAL STRENGTH: normal and symmetric

AU: clear

Procedure: Myringotomy with Tube

Indication: Eustachian tube dysfunction, not able to tolerate HBO

Procedure and attendant risks, benefits and alternatives discussed with patient. All questions answered. Consent signed.

Laterality: left

Patient placed under the microscope.

Phenol applied to TM.

Myringotomy incision created,

No middle ear fluid.

Tube placed: Armstrong-grommet

Patient tolerated the procedure well. No bleeding. No dizziness.

Assessment & Plan

Chronic left Eustachian tube dysfunction

He experiences significant pressure in the left ear, causing fullness, popping, and cracking, which have persisted for several months, affecting daily life and sleep. Previous treatments with nasal sprays and steroids were ineffective. An audiogram confirms Eustachian tube dysfunction. Due to chronicity and lack of response to medical management, tympanostomy tube placement was performed in the left ear. He was advised to keep water out of his ears for a few weeks post-procedure. After a couple weeks, swimming is allowed with the tube, but scuba diving is not. Potential risks of tympanostomy tube placement were discussed.

Chronic rhinosinusitis

Mr. May is being followed by Dr. Gray for his chronic sinusitis. He states he has been improved with use of budesonide rinses. He is out of refills. Additional refills prescribed.

F/u 3 weeks. Consider right PE tube next visit.

Gregory E. Krause, MD
Comprehensive Otolaryngology
Massachusetts Eye and Ear Infirmary